Fax cover sheet

Orange Grove General Hospital (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Derek Lowe, RN CM
Phone: 555-0160
Date: 06/08/2026 14:10
Pages (including cover): 12
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]

Message

52F R forearm abscess s/p I&D w/ MSSA bacteremia. PICC placed, cefazolin IV through 06/17.
Home health arranged. Hoping for d/c 06/09.

CONFIDENTIALITY NOTICE

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

ILLUMINATION Recuperative Care Referral Form

HEALTH + HOME

To refer a patient, please complete and submit this form with supporting documents via email to RECUP@ifhomeless.org or FAX to (888) 382-9551. For referral questions, please contact an Illumination Referral Coordinator at (888) 505-0855.

Referring Party

Referring Hospital: Orange Grove General Hospital (fictional) Dept/Floor: 2 West Medical
SW/CM/RN: Derek Lowe, RN CM Phone/Pager #: 555-0160
Email: dlowe@orangegrove.example Nursing Station #: 555-0161
Authorized by: Dr. M. Petrov Phone #:

Patient Information

Patient Name: Angela Doe MRN: OG-1175320 Date of Birth (dd/mm/yyy): 10/11/1973
Gender: ☐ Male ☒ Female ☐ Transgender ☐ Other Ethnicity: Asian (Vietnamese)
Last 4 # of Social Security: xxx-xx- 0000 Insurance: Orange Coast Health Plan (fictional) Medi-Cal Preferred Language: Vietnamese (interpreter for medical info)
Housing Situation: ☒ Currently Unhoused ☐ 4+ episodes of homelessness in the past 3 years

Hospital Admission Details

Chief Complaint Admitting Diagnosis:
R forearm abscess, MSSA bacteremia
Admit Date: 06/01/2026 Any Wounds? ☒ Yes ☐ No
If yes, explain the number, location, size and stage:
1 - R volar forearm I&D site 3 x 1.5 cm, packed
Independent with Wound Care? ☒ Yes ☐ No If no, will Home Health be ordered? ☒ Yes ☐ No
Was PPD/TB Test or Chest X-Ray Performed? ☒ Yes ☐ No If yes, Date: 06/01/2026 Results: CXR neg

History of Health & Living Situation

Does the patient require or have... (Please circle Y or N and provide explanation accordingly.)

O2	Yes* ☒ No	Active Infection	☒ Yes* No
*Explain		*Explain	MSSA - on IV cefazolin via PICC
Self Administer All Meds	☒ Yes No*	Ambulatory	☒ Yes No
Continent of Bowel & Bladder	☒ Yes No	Assistive Device	Yes* ☒ No
Colostomy/Ileostomy	Yes ☒ No	*Explain	
Foley Catheter	Yes ☒ No	Hospice	Yes* ☒ No
Diabetes	Yes ☒ No	*Explain	
Insulin	Yes ☒ No	Car	Yes ☒ No
Diabetes related Oral Meds	Yes ☒ No	Service Animal	Yes ☒ No

Behavioral and Mental Health Status

Does the patient have any limitations, behavioral or mental health challenges? ☒ No Yes
☐ Mental Health DX If checked, what type?
☐ Non-compliant ☐ Forgetful
☐ Cognitive Impairment ☐ Registered Sex Offender
☐ Other Explain

Length of Stay Authorized in Recupertive Care: 14 Days

ILLUMINATION STAFF ONLY

Date Received: Reviewed by:
Approved: ☐ Yes* ☐ No** *Date: *Authorization #: **Reason for Denial:
Admission Date: Admission Time: Exit Date:
Extension Request: ☐ Yes ☐ No If YES, Additional Approval Date: by Total LOS

CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)

Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Angela Doe

DOB: 10/11/1973

Member ID: OCH-64417702

Referring Provider

Facility: Orange Grove General Hospital (fictional)

Clinical

Primary Diagnosis: Cutaneous abscess of right upper limb; MSSA bacteremia

ICD-10: L02.413, R78.81, B95.61

Member is experiencing homelessness [X]

Member at risk of hospitalization/re-hospitalization [X]

Requested Start Date: 06/09/2026

Requested Days: 14

Plan Use Only

Status: Pending review

-- FICTIONAL DEMO RECORD - not a real patient --

Patient face sheet

Orange Grove General Hospital (fictional) - PATIENT FACE SHEET

Patient: Angela Doe
MRN: OG-1175320
Account #: 99175320
DOB: 10/11/1973
Sex: Female
Preferred Language: Vietnamese
Ethnicity: Not Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 06/01/2026 10:12
Admit Type: Inpatient
Unit/Bed: 2W-221
Attending: Mikhail Petrov, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed
Interpreter needed: Yes (Vietnamese)

-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical - page 1

HISTORY AND PHYSICAL

Date: 06/01/2026 (Vietnamese interpreter #4471 by video)

CC

R arm swelling and pain x 5 days, fever

HPI

52 y/o F, unhoused (sleeps in tent near Example Park), recycler, sustained puncture wound to R forearm from metal scrap ~1 wk ago. Progressive swelling, redness, pus, fevers to 102F. Denies IVDU.

ED: T 102.4F, HR 118, BP 104/62, WBC 18.9, lactate 2.6. Bedside US: 3.5 cm fluid collection R volar forearm.

PMH

- None known; no regular care

Meds

None

Allergies

NKDA

Social

No tobacco, no EtOH, no drugs. Daughter in Garden Grove, limited contact.

-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical - page 2

Exam

Gen: ill-appearing, AOx4 via interpreter
R forearm: 6 x 5 cm fluctuant area volar aspect with surrounding erythema 12 cm, no crepitus, full finger ROM, compartments soft
CV: tachycardic

Assessment/Plan

Sepsis due to R forearm abscess.
1. Bedside I&D in ED - purulent material sent for culture; packing placed
2. Blood cx x2, vancomycin IV + cefazolin IV pending cultures, IVF 30 mL/kg
3. Hand surgery consult - no deep space infection on exam; I&D adequate
4. Tetanus booster given
5. DVT ppx: enoxaparin 40 mg SC daily

Signed: Mikhail Petrov, MD 06/01/2026 13:20

-- FICTIONAL DEMO RECORD - not a real patient --

Infectious disease consult

INFECTIOUS DISEASE CONSULT

Date: 06/03/2026, follow-up 06/07/2026

Micro: Blood cx 06/01 2/2 MSSA. Wound cx 06/01 MSSA (cefazolin S, oxacillin S, TMP-SMX S).

Repeat blood cx 06/03 x2: no growth (final 06/08).

TTE 06/04: no vegetation, EF 60%.

Assessment: Uncomplicated MSSA bacteremia from skin/soft tissue source, cleared 06/03.

Recommendations:

- Cefazolin 2 g IV every 8 hours through 06/17/2026 (14 days from first negative cx)
- Vancomycin discontinued 06/03
- PICC OK to place (cultures negative >48h)
- Weekly CBC w/ diff, BMP while on IV abx - fax to ID clinic 555-0165
- ID clinic f/u 06/15/2026

Signed: L. Ramirez, MD (Infectious Disease)

-- FICTIONAL DEMO RECORD - not a real patient --

Progress note and case management note

HOSPITALIST PROGRESS NOTE - HD#8

Date: 06/08/2026 08:10

S: Feels well, arm pain 2/10. Afebrile since 06/03.

O: T 98.5F HR 80 BP 118/72. R forearm: erythema resolved to 2 cm periwound, I&D site 3.0 x 1.5 x 1.0 cm, packed.

PICC placed 06/07 L basilic, 4 Fr single lumen, tip at cavoatrial junction on CXR.

A/P: MSSA bacteremia cleared, on cefazolin IV.

- Medically ready for d/c once home infusion/HH confirmed
- PT consult ordered 06/07 - pending (not yet seen)

CASE MANAGEMENT NOTE

Date: 06/08/2026 12:45

Home infusion and HH referrals sent 06/07 to Bayside Home Infusion (fictional) 555-0167 and Coastline Home Health (fictional) 555-0131.

Status 06/08: Bayside reviewing - insurance auth pending. Coastline: awaiting response. NO agency acceptance yet.

Who will administer cefazolin q8h: TBD (agency RN vs teaching pt elastomeric pump - pending agency).

Will update IHH when confirmed.

Signed: Derek Lowe, RN CM

-- FICTIONAL DEMO RECORD - not a real patient --

Nursing note

NURSING NOTE

Date: 06/08/2026 06:30

PICC L arm: dressing C/D/I, flushes easily, + blood return. Dressing change due 06/14.

R forearm wound: packing removed and replaced by RN with 1/4 in iodoform strip, dry gauze. Moderate serosanguineous drainage.

Pt has NOT yet been taught wound packing; states "nurse does it". Pt hesitant to pack own wound.

Ambulating to bathroom and in room independently. Has not walked in hallway this shift.

Pt AOx4 via interpreter. Calm, cooperative.

Signed: H. Bui, RN

-- FICTIONAL DEMO RECORD - not a real patient --

Vital signs flowsheet

VITAL SIGNS - LAST 5 SETS

Date/Time	Temp (F)	HR	BP	RR	SpO2	O2
06/06 20:00	98.7	82	120/74	16	98%	RA
06/07 08:00	98.4	78	116/70	16	99%	RA
06/07 20:00	98.9	84	122/76	16	98%	RA
06/08 00:00	98.2	76	114/68	16	98%	RA
06/08 08:10	98.5	80	118/72	16	99%	RA

-- FICTIONAL DEMO RECORD - not a real patient --

Laboratory and microbiology

LABS

06/01 | WBC 18.9 H | Hgb 12.1 | Plt 402 H | Cr 1.1 | Lactate 2.6 H
06/03 | WBC 11.2 H | Cr 0.8
06/07 | WBC 7.9 | Hgb 11.6 L | Plt 365 | Cr 0.7 | AST 22 | ALT 18

MICRO

06/01 Blood cx x2: Staphylococcus aureus (MSSA) 2/2
06/01 Wound cx R forearm: MSSA
06/03 Blood cx x2: No growth, FINAL 06/08
06/02 MRSA nares PCR: Negative
06/08 07:00 SARS-CoV-2 rapid antigen: Negative

IMAGING

06/04 TTE: no vegetations, EF 60%
06/07 CXR: PICC tip at cavoatrial junction, lungs clear

-- FICTIONAL DEMO RECORD - not a real patient --

Discharge medication list (draft)

DISCHARGE MEDICATION LIST - DRAFT (not finalized)

Patient: Angela Doe MRN: OG-1175320

- Cefazolin 2 g IV every 8 hours via PICC through 06/17/2026 - administration by home infusion agency TBD
- Acetaminophen 650 mg PO q6h PRN pain
- Ibuprofen 400 mg PO q8h PRN pain with food

Wound supplies

- Iodoform packing strip 1/4 in, gauze, tape

Stopped

- Vancomycin IV (06/03)
- Enoxaparin (inpatient only)

-- FICTIONAL DEMO RECORD - not a real patient --